

Is Physical Therapy Safe From AI?

The already-in-it guide — the full scores by track, the six factors behind them, and what to do from where you already are.



2.5

AI EXPOSURE · PHYSICAL THERAPY (HANDS-ON)

where 10 is most at risk

For anyone already in it — in the degree, or job-hunting.



Before you read this

The choice is already made — you're in a physical therapy or allied health degree or training, or out of one and into the job hunt. So this guide skips the "should you study this" question and goes straight to the one that's actually live: given where you already are, what do you do about it?

It's written to you directly — whether you are the one in it, or a parent reading on their behalf. Where it says "you," a parent can simply read "them"; the moves are the same either way.

What this is. A facts document, not a pep talk. It sets out what's known about where this field sits in an AI economy, where the evidence is contested, and where we might be wrong — and then what to actually do from here.

The good news, up front. On the AI question, you're in one of the safest places in this entire index — hands-on clinical work scores 2.5, lower than anything we score except surgery. What's still in front of you isn't the automation risk; it's financial and directional, and almost all of it is still in your hands. Where you land — which setting, at what productivity cost, carrying what debt — is decided mostly by moves still ahead. The second half of this guide is those moves.

And one thing specific to this field, which matters more than anything else here. The AI question in physical therapy is about as settled as anything in this index. The financial question is not, and it is genuinely serious. The cost of a Doctor of Physical Therapy varies by more than \$300,000 between programs offering the identical qualification and leading to the identical career. If you're already committed to a program, some of that die is cast — but not all of it, and the financial section below sets out what's still in your control.

The short answer

Safe — and it is barely a close call.

Hands-on physical therapy scores 2.5 out of 10 for AI exposure — the lowest score of any career in this index apart from surgery. Occupational therapy scores 2.6, speech-language pathology 3.1.

The risk question here is effectively settled. What isn't settled is whether the qualification is worth what it costs — and that depends almost entirely on the program and the choices around it. So the work now is steering toward the protected, clinical end of the field and managing the financial side well, not worrying about being automated out.

The scores

ROLE	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Physical therapy (hands-on)	2.0	2.2	2.5	+0.5	Very low
Occupational therapy	2.1	2.3	2.6	+0.5	Very low
Speech-language pathology	2.6	2.8	3.1	+0.5	Low
Respiratory therapy	2.6	2.8	3.1	+0.5	Low
Diagnostic imaging technologist	3.5	4.0	4.4	+0.9	Low–Moderate
Records / screen-based allied roles	5.1	5.6	6.1	+1.0	Moderate–High

Scores run 1–10, where 10 is most at risk. For context: the median profession in this edition sits around 5.5, surgery scores 2.1, bedside nursing 2.8, and entry-level software development 8.1.

Read the table this way: the +0.5 movement on hands-on therapy is entirely administrative — documentation and exercise-plan drafting. The clinical work has not moved at all. The spread inside the field is the thing to hold onto: the same qualification, moved behind a screen, roughly doubles in exposure. Your job, from here, is to stay on the clinical side of that line.

Why it scores so low — the six factors



Here's how hands-on physical therapy answers them. Knowing *why* the clinical work scores 2.5 while the records-based roles sit at 6.1 is what tells you which way to steer.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

Less than almost any career we score.

Documentation and progress notes. Exercise plan drafting. Scheduling and billing. Outcome measure scoring. Insurance authorisation paperwork.

What resists: hands on a body, watching a gait in person, feeling resistance and guarding and compensation, motivating someone through months of painful work, and adjusting in real time to what a body actually does. Those are the things to keep at the center of your work.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Strong protection. Licensure requires supervised clinical hours, which cannot be automated or offshored, and demand is growing well above average as populations age and survival after serious illness improves.

But there's a fork worth seeing now, and it isn't about finding a job — it's about which version, and at what cost. The clinical, hands-on roles are the protected ones; the records-centered and screen-based allied roles that score 6.1 are not, and a seat there is the exposed rung however healthcare-adjacent it looks. The way through is to land on the clinical side — hands on a body, licensed, accountable — and to manage the debt around it deliberately, rather than drifting into a screen-based role because it's on offer, or assuming the outpatient mill with the highest starting number is the best first move. A protected career entered at the wrong price, or in the wrong seat, is still a hard financial decision.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

We rate this 9.5 — the highest physical rating we assign to any career. The protection-with-the-shortest-clock section below covers both why, and where we think it is most likely to change.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Yes. Rehabilitation is a relationship over weeks and months, and a licensed clinician carries duty-of-care obligations.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively. Licensure, scope of practice and supervision requirements are written into statute.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Frequently. Every body responds differently, and the judgment about when to push and when to stop is made in the moment. That judgment is the distance between the clinical work and the screen.

The protection with the shortest clock

We give physical therapy 9.5 on the physical factor — our highest rating anywhere. It does most of the work in a score of 2.5, and it deserves examining honestly, because of our four protections, physical is the one we rank least durable over a long horizon.

Why it's so strong today. Therapy is manual by definition: hands on a body, resistance felt rather than measured, adjustment in real time to how a person actually moves. Robotics lags software badly, and lags furthest in exactly this setting — an unpredictable human body, in a room, over months.

Why we flag it anyway. Someone qualifying now will practice for forty years. Over that span, licensure and human trust move at the speed of law and society and are unlikely to shift much. Physical protection moves at the speed of robotics, which is the one thing actively targeting it.

And there is early evidence worth taking seriously. Studies of robotic-assisted rehabilitation report meaningfully better motor outcomes in some domains than conventional therapy — not merely comparable, better. If robotic delivery is superior for certain protocols, "hands-on work is safe" starts looking less like a wall and more like a line.

Two things keep our score where it is. Better outcomes with robotic assistance is not the same as therapy delivered without a therapist — in every deployment we found, a licensed clinician prescribes, supervises and adapts. And the peer-reviewed literature is explicit that translation into routine practice remains limited.

The specific transition to watch is the same pattern that splits the skilled trades: whether robotic systems move from assisting a therapist to substituting for therapist hours in structured, protocol-driven rehabilitation. Structured settings are where automation always arrives first. Unstructured ones — a stroke patient at home, an athlete with an atypical injury — are last.

The financial question — and it's the real one

This is the section that should shape your next move, and it has nothing to do with AI.

The debt is substantial and well documented.

- The typical DPT graduate carries roughly **\$140,000–\$170,000** in total student debt at graduation
- Starting salaries in outpatient settings run **\$75,000–\$95,000**
- That is a debt-to-first-year-salary ratio of roughly 1.7 to 2.0

For comparison, the same analysis puts new physicians at 0.6–0.9x (much higher debt, much higher salary), nurse practitioners at 0.9–1.3x, pharmacists at 1.5–1.8x, and master's-level occupational therapists at 1.4–1.7x.

The APTA's own research is blunt about the consequences. Asked to give the word they most associate with student debt, physical therapists most frequently answered "burden," "unfair," "stressful" and "crushing." Around 28% of PTs report holding additional paid employment alongside their primary position, up from 24% in 2017. Separate industry survey data finds 84% of those with PT debt feel their salary is insufficient to manage the payments.

The number behind every debt decision

Analysis across **206 DPT programs at 151 institutions** found debt-to-income ratios ranging from **0.82:1 at the best program to 4.42:1 at the worst**. The median sits at 1.8:1.

That is a difference of more than \$305,000 in total cost for the same degree, the same license and the same career.

Worse, the federal loan cap makes the gap unavoidable at expensive schools. The mean annual funding gap across all 206 programs is around **\$33,647** above the federal borrowing limit — which over three years means roughly \$95,000–\$101,000 in private borrowing at an average-cost school. Private loans carry higher rates and, critically, **do not qualify for income-driven repayment or Public Service Loan Forgiveness.**

If you're already enrolled, the tuition is largely set — but the borrowing around it is not. Minimize additional and living-cost borrowing, exhaust federal options before private ones, and understand which of your loans qualify for income-driven repayment or Public Service Loan Forgiveness. Where the loan strategy still has room, that room is worth real money over the decades the debt lasts.

The alternative almost nobody is told about

The physical therapist assistant route.

- **Two-year associate degree** rather than a bachelor's plus a three-year doctorate
- Median pay around **\$65,510**, against roughly \$80,000–\$90,000 for a PT
- Dramatically lower debt — around \$24,000 on average where it exists at all

The arithmetic is striking. A PTA working through the years a DPT student spends in graduate school could have earned close to \$290,000 by the time the therapist qualifies — while the therapist starts with \$140,000–\$170,000 of debt.

Over a full career the DPT wins, and comfortably: the income difference compounds across thirty years and the ceiling is considerably higher. But the gap is smaller than families assume, it takes many years to close, and it depends heavily on which program was attended.

And on our scoring, the AI exposure is similar — PTA work is hands-on, physical and protected by the same things that protect the therapist.

This is not an argument against the DPT. It's an argument that someone drawn to the hands-on work rather than to autonomy and diagnosis should know the assistant route exists — and if you're early enough in the decision that it's still live, it's worth a serious look.

What the trend shows

Hands-on therapy: 2.0 → 2.2 → 2.5. Up 0.5 in three years — among the slowest movements in the index, and entirely administrative.

Records and screen-based allied roles: 5.1 → 5.6 → 6.1. Up 1.0.

The gap widened from 3.1 points to 3.6. The pattern holds exactly as everywhere else in this index: **move the same qualification behind a screen and exposure roughly doubles.** Which is why drifting into a screen-based allied role because it sounds modern is the one direction not to move.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Physical / embodied	Highest in the index	The one to watch	Robotic-assisted rehabilitation showing better outcomes in some protocols.
Regulatory / licensing	Strong	Most durable	Licensure and scope of practice move at the speed of legislation.
Human trust / accountability	Strong	Durable	Rehabilitation is a relationship sustained over months.
Judgment under novelty	Strong	Durable	Every body responds differently; the judgment is made in the moment.

The honest read: someone qualifying now will practice for forty years. Licensure and trust should hold comfortably. Physical protection is the strongest shield today and the one with the most credible long-term threat — and it is worth watching structured, protocol-driven rehabilitation specifically rather than the profession as a whole.

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing progress notes after every session	Reviewing a generated draft
Building the exercise program by hand	Adapting a generated program to this patient
Scoring outcome measures	Reading outcome data that arrives scored
Chasing insurance authorisation	Largely automated
Hands on a body	Unchanged
Knowing when to push and when to stop	Unchanged

The pattern here is unusually clean. In physical therapy the automatable layer and the clinical layer are cleanly separable, and what's automating is documentation — which the APTA's own research names among the primary contributors to burnout.

Human strengths that will matter most

1. **Manual assessment and treatment** — feeling resistance, guarding and compensation. The

least automatable thing in this index.

2. **Motivating someone through months of painful work** — most of rehabilitation is adherence,

and adherence is a relationship.

3. **Real-time judgment** — knowing when to push and when to stop, made in the moment with a

body in front of you.

4. **Accountability** — carrying duty of care for someone's recovery. 5. **Verification** — knowing when a generated program or note doesn't fit this patient.

Notably absent: protocol recall and documentation speed. These were traditional markers of a strong new graduate and are the fastest-depreciating. If your current value is mostly on that second list, the work is to shift it onto the first.

And underneath all five, two traits the score can't grade: **curiosity and adaptability**. The specific tools — the documentation system, the outcome-data platform, the robotic and sensor-assisted rehabilitation rigs — will turn over many times across a career. The clinicians who compound are the ones who stay genuinely curious about how a body recovers and re-tool fastest, rather than defending the one way of working they trained on. In a field where the physical protection is real today but has the shortest clock, that flexibility is itself a form of protection — and understanding where the new technology helps and where it misleads is a competency almost nobody is teaching yet.

Other things to consider about this job

What's genuinely good about it

Demand is strong and demographically driven rather than cyclical. Physical therapists are projected to grow around 11% and occupational therapists 14% through 2034 — both far faster than average — driven by an ageing population and improving survival after serious illness.

What therapists report as rewarding is unusually consistent:

- **Visible progress** — you watch someone recover across weeks in a way few jobs offer
- **Relationships that run over months**, not minutes
- **Autonomy**, with private practice a realistic destination
- **Physical, active work** rather than a desk
- **Genuine variety** across settings — hospital, outpatient, sports, paediatric, home health, neurological

And the career is sustainable into later life with specialization, and portable across settings and geographies.

What's genuinely hard about it

The debt, covered in the financial section above, and it is the dominant issue.

Burnout is high and the causes are not what you'd expect. Around 70% of physical therapists report moderate to high burnout, and the most-cited drivers are unrealistic productivity demands, heavy documentation requirements and administrative load — not the clinical work itself.

That's worth noticing, because it's precisely the layer that AI is removing. This may be the career in the whole index where automation most directly addresses the profession's actual complaint.

The work is physically demanding over decades. Standing, lifting, transferring patients.

And some patients do not improve, which is harder to sit with than students expect.

Who this work suits — and who it doesn't

Physical therapy tends to suit people who:

- **Want to see progress they can measure** — the single most-cited satisfaction
- **Are comfortable with sustained physical contact** and with bodies generally
- **Can motivate people who don't want to be motivated** — most of the job is adherence
- **Prefer moving to sitting**
- **Are patient with slow improvement**

It's harder for people who:

- **Need quick resolution** — rehabilitation is measured in months
- Find **repetitive documentation** draining, though this is the automating part
- Want **high early earnings** relative to the training cost
- Are uncomfortable when **someone doesn't get better**

If some of that second list sounds like you, it isn't a reason to abandon the path — it's a reason to steer toward the settings and seats that fit, which is what the rest of this does.

What else is moving this market

Demographics are the dominant force, and they are durable rather than cyclical.

Non-opioid pain management has increased referrals to physical therapy meaningfully — a policy shift rather than a technological one.

Productivity pressure is the profession's structural complaint, particularly in outpatient settings where reimbursement models push toward higher patient volumes per therapist.

And employer loan assistance is emerging but rare — around 80% of PTs and OTs carry student debt while only about 8% receive any employer repayment support. **Where it exists it is worth several years off a payoff timeline, and it is a legitimate thing to ask about at interview.**

The AI-native advantage — what it actually looks like

The situation: AI in physical therapy is arriving as documentation relief rather than clinical substitution, which makes this one of the more straightforwardly positive pictures in the index. Being already in it, close to the work, is a real advantage here — you can build this fluency inside a live placement or first job, which is exactly where it counts.

WHAT AN AI-NATIVE THERAPIST LOOKS LIKE

- 1. Reclaim the time deliberately.** Documentation automation only helps if the hours saved go somewhere. **Therapists who are intentional about this get more patient contact; those who aren't watch productivity targets absorb it.**
- 2. Verify.** Knowing when a generated note or program doesn't fit this patient — a clinical skill rather than a technical one.
- 3. Work with robotic and sensor-assisted rehabilitation.** These systems are arriving. Understanding where they help and where they mislead is a genuinely new competency and almost nobody is teaching it.
- 4. Use outcome data properly.** Continuous movement and adherence data is becoming available. Interpreting it clinically is a real skill.
- 5. Contribute to how it's implemented.** Services are writing these policies now, largely without practitioner input.

All five are available to you now — not after some future qualification. Turning them into concrete moves is the rest of this guide.

Where these qualifications can lead later

Specialization (orthopaedic, neurological, paediatric, sports, geriatric), private practice ownership, clinical education and academia, research, healthcare management, ergonomics

and occupational health, and industry roles in devices and rehabilitation technology.

The pattern in this index holds: the further from hands-on patient contact, the higher the exposure. Records-centered and utilization roles score 6.1 against clinical work at 2.5. That's the whole map, and it's the compass for every choice below.

Repositioning from here

The program-selection stage is behind you, or nearly. The useful questions now are different — and the good news bears repeating: on AI exposure, almost nothing about your position is at risk. The clinical work scores 2.5. What's still in front of you is financial and directional: which setting, at what productivity cost, and how well you manage the debt. Almost all of it is still in your hands.

If you're still in the degree or training

Three levers, in order of leverage.

1. Protect the clinical core. Manual therapy and assessment — feeling resistance, guarding and compensation — is the 2.5 work. The records-centered and screen-based allied roles are the 6.1. Take the hands-on modules and manual-therapy training as seriously as the score says they deserve, and get clinical breadth across as many settings as you can, because the setting shapes the career more than the qualification does.

2. Borrow as little more as you can, while the choices are still open. The tuition may be set, but the borrowing around it isn't. The federal loan cap leaves a mean funding gap of around \$33,647 a year at an average-cost school — roughly \$95,000–\$101,000 in private loans over three years — and private loans don't qualify for income-driven repayment or Public Service Loan Forgiveness. Minimize additional and living-cost borrowing now, exhaust federal options before private ones, and know which of your loans qualify for forgiveness.

3. Get fluent in rehabilitation technology while it's still free. Robotic and sensor-assisted rehabilitation (already showing better outcomes in some protocols) and continuous outcome data are arriving. Knowing where they help and where they mislead is the new competency, almost nobody is teaching it, and it's far cheaper to build in training than to bolt on later.

If you've graduated and you're job-hunting

Here the setting and the seat matter more than they look, because the field's real variation isn't in the automation risk — it's in what the job does to your finances and your burnout.

- **Aim at the clinical, hands-on roles (2.5), not the records-centered or utilization ones (6.1).** Direct patient care in PT, OT, SLP or respiratory therapy is the protected work; a screen-based allied seat is the exposed rung however healthcare-adjacent it looks.
- **Ask about productivity expectations and employer loan repayment assistance before you accept.** Only about 8% of PTs and OTs get any repayment support, but where it exists it's worth several years off the payoff — and around 70% report burnout driven by productivity quotas and documentation, not the clinical work. A higher starting number in a documentation-heavy outpatient mill can cost more than it pays.
- **Choose the setting deliberately.** Hospital, outpatient, sports, paediatric, home health, neurological are genuinely different jobs; the setting shapes the career more than the qualification does.
- **Prefer the seat with more patient contact.** That's where the satisfaction and the protection both live.

The AI-native move, from where you stand

You can't undo the tuition already committed, but you can be the clinician who directs these tools rather than lets them absorb your day. The five capabilities above — reclaim the time, verify, work with robotic-assisted rehab, use outcome data, contribute to implementation — are all available right now, inside a current placement or a first job. The most powerful for someone already in it is the first: be deliberate about where the hours documentation automation saves actually go. Reclaimed for patient contact, they compound your protection; left unmanaged, productivity targets absorb them. And being the practitioner who understands the new rehabilitation technology puts you in the room when services write the policies they're currently writing without practitioner input — a position you can start building toward this term, not after some future qualification.

The career-hunting checklist

The analysis tells you where the exposure is. This is what to do about it — the part that's actually in your hands. Most of the protection here is built in these choices and habits, rather than handed to you. The first list is for anyone still in the degree or training; the second is for the job hunt itself.

During the degree / training — build what can't be automated

- **Protect the clinical core.** Manual therapy and assessment — feeling resistance, guarding and compensation — is the 2.5 work. The records-centered and screen-based allied roles

are the 6.1. Take the hands-on modules as seriously as the score says they deserve.

- **Minimize the borrowing you can still control.** The federal loan cap leaves a mean funding gap of around \$33,647 a year at an average-cost school — roughly \$95,000–\$101,000 in private loans over three years, and private loans don't qualify for income-driven repayment or Public Service Loan Forgiveness. Exhaust federal options first and keep living-cost debt down.
- **Get clinical breadth across as many settings as possible.** Hospital, outpatient, sports, paediatric, home health, neurological — the setting shapes the career more than the qualification does.
- **Take rehabilitation technology seriously.** Robotic and sensor-assisted rehabilitation and continuous outcome data are arriving; knowing where they help and where they mislead is the new competency, and almost nobody is teaching it.
- **Get fluent with AI documentation as a clinician, not a shortcut.** Be able to verify — to know when a generated note or program doesn't fit this patient. That is a clinical skill, and it's the one that survives.
- **Feed curiosity and adaptability on purpose.** The tools will turn over repeatedly; the clinician who re-tools fastest, and stays genuinely curious about how a body recovers, compounds.
- **Keep the frame wide.** If the decision is still live at all, know that the assistant routes (PTA, OTA), occupational therapy, speech-language pathology and respiratory therapy reach similar protected work — several at a fraction of the debt.

During the job hunt — aim, don't just apply

- **Target the clinical, hands-on lanes on purpose.** Direct patient care in PT, OT, SLP or respiratory therapy — not records-centered or utilization roles, which are the 6.1 rung however healthcare-adjacent they look.
- **Read the role, not just the title.** A screen-based "records" or "utilization" allied role is the automating layer (6.1). If you take one, take it as a deliberate stepping-stone with an exit in mind, not as the plan.
- **Don't grab a role just because it's on offer in an exposed lane.** A role being available doesn't make it safe. An employer still building out a large screen-based allied or records team may simply be behind the curve — and that's exactly the rung that gets cut first once they catch up. Availability can be a lagging indicator, not a green light.
- **Ask about productivity expectations and employer loan repayment assistance before accepting.** Only about 8% of PTs and OTs get any repayment support, but where it exists

it's worth several years off the payoff — and both these things shape the first five years more than the title does.

- **Prefer the setting and seat with more patient contact.** Around 70% of therapists report moderate to high burnout, driven by productivity quotas and documentation rather than the clinical work — so weigh a lower-quota clinical seat over a documentation-heavy outpatient mill.
- **Ask the employer where AI and rehabilitation technology are heading in the role.** Whether they're building the job around documentation relief and robotic-assisted rehab or waiting to be forced to tells you if the seat grows or is quietly being hollowed out.
- **Weigh the trajectory, and the debt math, over the starting salary.** The financial decision in this field is made role-by-role, not by chasing the highest first-year number into a high-burnout seat.

Go deeper — further reading

- **Physical Therapists — Occupational Outlook Handbook** — US Bureau of Labor Statistics. Primary source and free: projected growth, median pay, entry requirements. Read the **Physical Therapist Assistants** page alongside it — the comparison is the most useful twenty minutes available on this decision.
- **Impact of Student Debt on the Physical Therapy Profession** — American Physical Therapy Association. The profession's own research into what the debt is doing to its members, including the finding that 28% hold additional employment. Read it if you're weighing how hard to push on loan strategy.
- **Is physical therapy worth the debt? 2026 ROI analysis** — the program-by-program debt-to-income analysis across 206 DPT programs. Useful context for understanding where your own numbers sit.

The counterargument — read this one:

The strongest case against our 2.5 isn't that AI will replace therapists. It's that a very low AI score may lead someone to under-weight a genuinely serious financial risk.

On this reading, our framework says physical therapy is among the safest careers available — which is true — while saying nothing about a debt-to-income ratio that can reach 4.42:1 at the wrong program, private borrowing that doesn't qualify for forgiveness, and 84% of indebted PTs reporting their salary is insufficient to manage the payments.

A career can be automation-proof and still be a hard financial position.

Where we land: this is entirely fair, and it's why the financial section sits before the "other considerations" section rather than inside it. Our score measures automation exposure only. For physical therapy specifically, the AI answer is genuinely reassuring and the financial answer depends almost entirely on the program and the choices around it — and conflating the two would be the most damaging thing this document could do.

Bottom line

On AI exposure, this is about as good as it gets — which is exactly why being already in it is not a problem. Hands-on physical therapy scores 2.5 — lower than anything in this index except surgery — protected by manual work, licensure, trust and genuine unpredictability. Demand is demographically driven and durable. And the automation that is arriving removes documentation, which the profession's own research names among the leading causes of burnout.

The real decision from here is financial and directional, and it is specific to an extraordinary degree. Debt-to-income ratios across 206 DPT programs run from 0.82:1 to 4.42:1 — a spread of over \$305,000 in total cost for the same degree and the same career. Where your own numbers sit is partly set and partly still in your control.

So the useful advice isn't a verdict on the path — it's a direction. Protect the clinical core and get breadth across settings. Manage the borrowing you can still control, and know which loans qualify for forgiveness. Aim the job hunt at clinical, hands-on seats over screen-based allied roles, and ask about productivity and loan repayment before you accept. And build real fluency with documentation automation and rehabilitation technology now, inside whatever role you're in, because that's the differentiator this field is about to reward.

And one thing worth saying that no score captures. Watching someone walk again, or lift their arm again, or return to something they thought they'd lost — very few jobs offer that at all. The financial caution above is real and it is about *which* choices, not about *whether* you're in the right field.

Questions worth sitting with

Whether you're the one in it or a parent thinking it through, these are the conversations that move the needle now. Read "you" as whoever the decision is about.

Where you actually stand

1. Which track are you on — or aiming at — and does it point at the clinical, *hands-on* work (2.5) or at a *records-centered* / *screen-based* allied role (6.1)? 2. Looking at the score table, which destination are you currently pointed at, and is it the one you'd choose on purpose? 3. What does your own debt picture actually look like — how much is federal, how much private, and do you know which of it qualifies for income-driven repayment or forgiveness?

The move from here

4. What's the single most protected, most satisfying setting you could realistically aim for from where you are now, and what would make you a credible candidate for it? 5. If you're still studying: is there any borrowing or living-cost debt still in your control that you could bring down before it's locked in? 6. If you're job-hunting: are you aiming at a clinical, hands-on seat on purpose, or applying to whatever's open? Have you asked about productivity expectations and loan repayment assistance?

The AI question, practically

7. Of the five capabilities — reclaim the time, verify, work with robotic-assisted rehab, use outcome data, contribute to implementation — which one could you visibly demonstrate within the next few months? 8. When documentation automation frees up time in your day, where does it actually go — more patient contact, or absorbed by productivity targets? Have you a plan for that?

Testing it against reality

9. **The most valuable single action:** shadow or talk to a therapist in two very different settings — outpatient orthopaedic and something like neurological or paediatric — and ask directly about their loan payments and whether they'd choose the same first job again. 10. If the first offer that comes is a documentation-heavy, high-quota outpatient role, what's your plan — take it as a deliberate stepping-stone with an exit in mind, or hold out for a setting with more patient contact? Decide before the offer, not after.

This is analysis and informed judgment about a fast-moving situation. For physical therapy specifically, we've flagged that the profession's most serious risk is financial rather than technological, and that it varies enormously by program and by choice — something our six factors cannot see.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Physical Therapy & Allied Health

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

$$\begin{aligned} \text{Risk} = & 0.35 \times \text{Automatability} \\ & + 0.15 \times \text{EntryErosion} \\ & + 0.15 \times (10 - \text{Physical}) \\ & + 0.15 \times (10 - \text{Trust}) \\ & + 0.10 \times (10 - \text{Regulatory}) \\ & + 0.10 \times (10 - \text{Judgment}) \end{aligned}$$

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-eight careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human?*

RATING	ANCHOR
0–2	No license exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent, where being wrong is costly?*

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

PHYSICAL THERAPY (HANDS-ON) — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.1	4.5	5.2	1.82
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.5	9.5	9.5	0.07
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	2.49 → 2.5

OCCUPATIONAL THERAPY — 2.6 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	5.5	1.92
Entry-path erosion	15%	1.2	1.5	1.8	0.27
Physical / embodied	15%	9.4	9.4	9.4	0.09
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	2.61 → 2.6

SPEECH-LANGUAGE PATHOLOGY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.2	5.7	6.4	2.24
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	8.5	8.5	8.5	0.22
Human trust / accountability	15%	9.3	9.3	9.3	0.1
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.5	8.5	8.5	0.15
				Total	3.09 → 3.1

RESPIRATORY THERAPY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.1	5.6	6.3	2.2
Entry-path erosion	15%	1.5	1.8	2.0	0.3
Physical / embodied	15%	8.8	8.8	8.8	0.18
Human trust / accountability	15%	9.0	9.0	9.0	0.15
Regulatory / licensing	10%	9.3	9.3	9.3	0.07
Judgment under novelty	10%	8.2	8.2	8.2	0.18
				Total	3.08 → 3.1

DIAGNOSTIC IMAGING TECHNOLOGIST — 4.4 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	6.1	7.0	2.45
Entry-path erosion	15%	2.5	3.0	3.4	0.51
Physical / embodied	15%	7.5	7.5	7.5	0.38
Human trust / accountability	15%	7.0	7.0	7.0	0.45
Regulatory / licensing	10%	8.5	8.5	8.5	0.15
Judgment under novelty	10%	5.5	5.5	5.5	0.45
				Total	4.38 → 4.4

RECORDS / SCREEN-BASED ALLIED ROLES — 6.1 (MODERATE-HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.6	5.9	7.1	2.48
Entry-path erosion	15%	4.5	5.0	5.5	0.82
Physical / embodied	15%	1.5	1.5	1.5	1.27
Human trust / accountability	15%	5.5	5.5	5.5	0.67
Regulatory / licensing	10%	7.0	7.0	7.0	0.3
Judgment under novelty	10%	4.5	4.5	4.5	0.55
				Total	6.11 → 6.1

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Physical therapy (hands-on)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.1	1.2	0.4	2.0
2025	4.5	1.5	0.4	2.2
Fall 2026	5.2	1.8	0.4	2.5

Movement decomposition: automatability contributed **+0.39** and entry-path erosion **+0.09**, for a total of **+0.5** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labor market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.